

OBSESSIVE COMPULSIVE DISORDER

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Published in June 2024. Next review date: June 2027.

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INTRODUCTION

Obsessive and compulsive features may be an appropriate adaptive response to a stressor, especially in occupations or situations whereby attention to detail and order are required as part of the job. However, Obsessive Compulsive Disorder (OCD) is an exaggeration of this response such that it causes social and/or occupational dysfunction.

Example of a patient with OCD

Mr A drives to work frequently. He is troubled by repeated thoughts and anxiety that he has accidentally hit a pedestrian while driving. He gets these thoughts throughout the day uncontrollably, whether while driving or after parking his car and starting work. He copes by repeatedly driving to and from his workplace and home via the same route to check if he has caused an accident or if anyone was hurt. He does this several times for his drive to work and this results in him taking 2 hours to get to work instead of the average of 20 minutes.

SCREENING AND DIAGNOSIS

What It Is (DSM-5)

OCD is indicated by the presence of obsessions (unwanted, repeated, intrusive thoughts or images that bring about distress, which the person tries to counteract and alleviate with other thoughts or actions) and/or compulsions (repetitive ritualized actions that the individual is attempting to counteract the thoughts, urges or images, and/or are excessive in their application).

Obsessions and/or compulsions are time-consuming, e.g., taking more than 1 hour/day, or cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.¹ Common obsessive themes revolve that about hygiene, cleanliness, orderliness, sex, and religion.²

These disturbances are not attributable to effects of a substance or another medical condition, and cannot be better explained by symptoms of another mental disorder, for example:

1. Body dysmorphic disorder
2. Hoarding disorder
3. Trichotillomania
4. Excoriation (skin picking) disorder
5. Generalised anxiety disorder

6. Ritualised eating behaviour / eating disorders
7. Gambling disorder
8. Illness anxiety disorder
9. Substance / medication induced obsessive – compulsive related disorder

The lifetime prevalence of OCD is 1.6% for which there is a strong genetic / biological influence.²

The main distinguishing factor between obsessive compulsive disorder and obsessive compulsive personality disorder is the presence of anxiety in the former as compared to the latter.² These individuals range from having good insight, where they recognize that the obsessive-compulsive disorder beliefs are definitely or probably not true, to absent insight or delusional beliefs, where individual is completely convinced that obsessive-compulsive disorder beliefs are true.

When It Is Significant

OCD becomes an issue when obsessions and/or compulsions are so time consuming that they cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

Most psychiatrists go by clinical assessment to determine the impact of OCD symptoms on patients' lives. If you wish, you may use the Yale – Brown Obsessive Compulsive Scale (YBOCS) available as part of Epic Flowsheets if time permits, as part of your evaluation of the complaint. Interpretation of YBOCS is as follows:

Total score	Interpretation
0-7	Subclinical symptoms of OCD
8-15	Mild symptoms of OCD
16-23	Moderate symptoms of OCD
24-31	Severe symptoms of OCD
32-40	Extreme symptoms of OCD

For those with OCD regardless the severity, do proceed to the following interventions that could help, as described below.

MANAGEMENT

General Overview

Non-Pharmacological

Cognitive behavioural therapy / Exposure and Response Prevention Therapy (CBT/ERP)

Pharmacological

1. SSRI

- a) Fluoxetine
- b) Fluvoxamine
- c) Sertraline
- d) Escitalopram
- e) Paroxetine

2. TCA

- a) Clomipramine

* Less effective: mirtazapine, bupropion or agomelatine

NON-PHARMACOLOGICAL

Management	How does it work and what is involved
CBT/ERP	CBT with an emphasis on ERP is the recommended mode of management for OCD • ERP: therapist guided repeated and prolonged exposure to situations that provoke obsessional fear along with abstinence from compulsive behaviours. • ERP requires the patient remain in the exposure situation until the obsessional distress decreases spontaneously, without the patient having to reduce distress by withdrawing from the situation or by performing compulsions to cope with the obsessional thoughts. <i>The goal of ERP is to challenge how a patient responds to distress and to eventually learn that feared stimuli are safe.</i> • <i>Patients can be exposed to actual situations (in vivo exposure), imagined situations (imaginal exposure), or the physical sensations associated with anxiety or discomfort (interoceptive exposure)</i> i.e., for the patient described in the introduction who fears accidentally hitting pedestrians while driving will practice driving on streets with pedestrians around, without getting out of the car to check the roadside for victims. The frequency of CBT with ERP recommended varies depending on the severity of the OCD, ranging from once a week outpatient psychology appointments to even daily psychology outpatient appointments.³ An example of daily psychology outpatient treatment is the Bergen 4 Day Treatment (B4DT) that is based on ERP basic principles. This is only available in certain centres.⁴

PHARMACOLOGICAL

Patients with OCD should be treated with ERP and pharmacological management.

The average maintenance dose required to treat patients with OCD is higher compared to patients with MDD or GAD. Majority of patients with OCD will continue to have residual symptoms despite maximal dose of tolerable medications. In addition, patients tend to take longer to achieve remission after initiation of psychotropics, hence patients tend to be on psychotropics for a longer duration for OCD as compared to other anxiety disorders.

Although it is possible to wean off psychotropics, it is considerably more difficult in patients with OCD. In general, patients with OCD are put on long-term medications as the risk of relapse is high without medications. This means that patients end up having their dosage of medications reduced to the low range of the maintenance dosing e.g., fluoxetine 60mg daily. The aim of treatment is for patients to be able to cope with some residual symptoms and have good function in life.

Drug	Usual Dose Range	Common ADR (Adverse Drug Reactions)	Remarks / Contraindications / Precautions
Selective Serotonin Reuptake Inhibitors (SSRIs)			
Fluoxetine (S2) \$\$^	Initial: 10-20 mg/day Maintenance: 40-80 mg/day (up to 120 mg/day)	• Gastrointestinal e.g., nausea, diarrhoea • Insomnia (more common in Fluoxetine, Sertraline) • Somnolence (more common in Fluvoxamine) • Nervousness • Headache • Dizziness • Sexual dysfunction • Weight gain	• Avoid abrupt discontinuation, taper dose over several weeks. • May cause hyponatremia/ SIADH, serotonin syndrome, QT-prolongation, bleeding, activation of mania or hypomania. Avoid in angle-closure glaucoma. • Caution in seizure disorders, renal and hepatic impairment. • Higher doses are associated with increased ADR. • Contraindicated within 2 weeks of MAOI use. • Escitalopram and Sertraline have less drug-drug interactions.
Fluvoxamine (S2) \$\$^	Initial: 50 mg/day Maintenance: 100-300 mg/day (Divide total daily doses >100 mg into twice-daily dosing)		
Escitalopram (NS) \$\$\$\$\$^	Initial: 10 mg/day Maintenance: 20-40 mg/day (up to 60 mg/day)		
Sertraline (S2) \$\$\$^	Initial: 50 mg/day Maintenance: 200 mg/day (up to 400 mg/day)		

Drug	Usual Dose Range	Common ADR (Adverse Drug Reactions)	Remarks / Contraindications / Precautions
Tricyclic Antidepressants			
Clomipramine (S1) \$^a	Initial: 25 mg ON Maintenance: 100-250 mg ON	- Somnolence - Weight gain - Anticholinergic effects (dry mouth, constipation) - Dizziness - Headache	- Avoid abrupt discontinuation, taper dose gradually. - May cause hyponatremia/ SIADH, serotonin syndrome, QT-prolongation, bleeding, activation of mania or hypomania. - Avoid in the elderly and in angle-closure glaucoma. - Caution in seizure disorders, renal and hepatic impairment. - Contraindicated within 2 weeks of MAOI use. - Less favoured overall in view of risks of toxicity with overdose.

^a Cost \$ <\$10 per month \$\$ \$10 - <\$20 per month \$\$\$ \$20 - <\$30 per month
 \$\$\$\$ \$30 - <\$60 per month \$\$\$\$\$ \$60 - <\$90 per month \$\$\$\$\$\$ \$90 or more per month

Amount payable depends on patient subsidy level and drug subsidy class (Standard Drug List S1 and S2, Non-Standard Drug NS).

Unit prices before GST and accurate as of Mar 2024.

Prices shown are an estimate. Please contact pharmacy for accurate pricing.

De-Escalation of Therapy

If shared decision is to trial de-escalation (this scenario should be rare), gradually reduce the dosage of the patient's psychotropics. Arrange a review 2-3 months after each reduction in dosage to look for recurrence of symptoms.

Recommended maintenance dosage of psychotropics in management of OCD

Name of Medication	Dosage
Fluoxetine	60 – 80 mg daily
Fluvoxamine	150 – 200 mg daily
Sertraline	150 – 200 mg daily
Escitalopram	20 mg daily
Clomipramine	150 – 200 mg daily

Management of Step-Down Patients from NUHS / IMH Psychiatry SOC to Polyclinic

All newly diagnosed patients with OCD should be referred to Psychiatry SOC's for initiation of treatment and more intensive psychotherapy. Once patient is stable (at least 6 months on the same dose of medications with acceptable level of residual symptoms, and no longer needing psychotherapy), then NUHS / IMH Psychiatry SOC's can right-site these patients to the nearest or enrolled NUP polyclinics to continue management of these patients.

If patients are not enrolled to HSG but are eligible, please enrol HSG and empanel teamlet.

When the admin doc vets the step-down referrals, these patients should be given a 'TCU Dr Health and Mind consult' visit type, and not the TCU Dr health and mind consult (LONG FIRST VISIT), as these patients are review patients and not first visits in the mental health ecosystem. If stable these patients can be decanted to specific teamlet (not to general pool, as there is no defaulter scrubbing for patients in general teamlet).

What To Do at HMC Or Teamlet When Seeing Step Down Stable OCD Patients

Doctor is to:

1. Assess severity of OCD

- a) Assessing degree of dysfunction OCD has caused to social / occupational / family life
- b) Doing a mental state examination

Doctor can consider using Y-BOCS on Epic Flowsheet (items 1-10 are scored, items 11-18 is optional) as an objective measure of severity of OCD.

2. Assess for risk of self-harm / harm to others.



Mild OCD



Continue the same medications.

TCU 3-6 months
Decant to teamlet at 2nd HMC visit.

If mild relapse of symptoms, can consider referring to NUP psychologists and KIV increasing the dose of medications.



Moderate OCD



Refer NUP psychologist to see within 1 month.

Uptitrate medications.
TCU patient to NUP HMC clinic within 1-2 months for follow up.

Consider TCU back to teamlet once Y-BOCS score is < 8.



Severe OCD or if there is risk of self-harm / harm to others



Do suicide assessment risk. Consider referring to ED if symptoms are clinically significant or patient possesses significant risk of self-harm.

Uptitrate medications, give 4 weeks supply.

Refer original psychiatry SOC stepped down from– direct access, AND email where relevant:
ASCAT NTFGH: JHCampus_CMH_programme@nuhs.edu.sg
ASCAT NUH: nuhascat@nuhs.edu.sg

Stepdown OCD Workflow

OCD on stable doses of oral medications for past 6 months with Psychiatry SOC.
Does not require active psychotherapy.

Psy SOC to check if HSG-enrolled clinic is in MHGPP list. If yes, step down to MHGPP if appropriate.

Fulfills the following criteria:

- No comorbid personality disorder.
- Medication must be part of NUP Formulary.
- Does not require long-term benzodiazepines or Z-drugs.
- Agreeable to be followed up at NUP polyclinics (patient to choose preferred polyclinic).

Information required from psychiatrist:

1. Whether patient is of higher needs, e.g., comorbid anxiety / depression.
2. Frequency of review when patient presents for stepdown care.

Psychiatrist orders 'referral to polyclinics – XXX polyclinic'
Instructs patients to check on OneNUHS App / HealthHub in 2 weeks' time and reschedule early if date does not suit.

NUP polyclinic admin doctor vets step down

Teamlet patient?

Yes

No

First visit to be seen at HMC as a **review case**, then if stable after 2 visits, step down to own teamlet

First visit to be seen at HMC as a **review case**, then if stable after 2 visits, recruit to teamlet

* If there is ongoing psychologist appointment in NUHS Psychiatry department, that arrangement is to continue

REFERRALS

A referral back to original psychiatry SOC is warranted for the following:

1. Severe OCD or if there is a risk of self-harm / harm to others
2. OCD not responding to optimal treatment with combination of ERP and an optimized dose of psychotropics
3. OCD requiring escalating doses of benzodiazepines (follow latest benzodiazepine prescription guidelines for primary care)
4. Patient assessed by NUP psychologist to require intensive ERP such that NUP psychology is not able to support
5. Patients with concomitant drug or substance abuse
6. All newly diagnosed patients with OCD

ABBREVIATIONS

OCD: Obsessive compulsive disorder
GAD: Generalised anxiety disorder
MDD: Major depressive disorder
MSE: Mental state exam
ASCAT: Assessment and shared care team

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